

Clinical Research Coordinator Training

Module 3: Informed Consent

SYNTHETIC TRAINING MATERIAL - NOT A REAL PROTOCOL

Written as a demo fixture for the CADA annotation platform.

Nothing in this document describes an actual study, site, sponsor, or patient.

3.1 Purpose

Informed consent is a continuing conversation, not a signature event. The coordinator confirms that a prospective participant understands what the study asks of them, what is known about the risks, and that taking part is voluntary.

3.2 Required elements

- a) A statement that the study involves research
- b) The expected duration of participation
- c) A description of procedures, marking which are experimental
- d) Reasonably foreseeable risks and discomforts
- e) Benefits that may reasonably be expected
- f) Alternative procedures or courses of treatment
- g) How confidentiality of records will be maintained
- h) Whom to contact with questions about the research
- i) That refusal to take part involves no penalty

3.3 Assessing comprehension

Ask the participant to describe the study back in their own words. Closed questions such as 'Do you understand?' reliably produce agreement whether or not comprehension occurred.

Teach-back prompts that work well:

- What will you be asked to do at each visit?
- What happens if you decide to stop?
- Which part of this is different from usual care?

3.4 Reading level

Consent forms should be written at roughly an 8th-grade reading level. Common substitutions:

- randomized -> assigned by chance, like a coin toss
- adverse event -> a health problem during the study
- placebo -> a look-alike with no medicine in it
- discontinue -> stop

Readability scores are a guide, not a target. A form can score well and still be unclear if it hides the burden of the study.

3.5 Documentation

Record the consent conversation in the participant's record: who presented the study, the version of the form used, the date and time, and any questions that were asked.

The signed form is filed in the regulatory binder. The participant keeps a copy.

3.6 Re-consent

Re-consent is required when new safety information emerges, when the protocol changes what participants are asked to do, or when a minor reaches the age of majority during the study.

3.7 Common findings in monitoring visits

- Form signed after the first study procedure
- An outdated form version in use at one site
- Missing date written by the participant
- No note describing the consent discussion

End of Module 3.